HMG-CoA (3-hydroxy-3methylglutaryl-coenzyme-A) reductase inhibitors, also called 'statins' are used primarily to reduce low-density lipoprotein (LDL) levels in the blood. They also reduce vascular inflammation and stabilize atherosclerotic lesions. The current practice is to institute 'statin' therapy immediately after a coronary event regardless of LDL levels. Examples include lovastatin, atorvastatin, fluvastatin, pravastatin, simvastatin, and rosuvastatin.

All of the 'statin' drugs undergo first-pass hepatic extraction. Their administration can result in three-fold increases in serum aminotransferase levels, but does not often produce symptoms of hepatic toxicity. Patients who do develop hepatic toxicity typically present with symptoms such as malaise, anorexia, and sharp declines in LDL levels.

Aminotransferase levels should be measured at baseline, at 1-2 months of therapy, and then every 6 to 12 months thereafter. These drugs are contraindicated in women who are pregnant, lactating, or likely to become pregnant. Creatine kinase levels may also become elevated, and may be associated with myalgias.

Katzung BG, Masters, SB, & Trevor AJ. Basic and Clinical Pharmacology. 12th ed. New York: McGraw-Hill; 2012: 625-627.

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